DRUGS · ALLERGY, RHINITIS AND BIOLOGICS

# Oral decongestants

Oral decongestants (pseudoephedrine, and the less effective phenylephrine) constrict nasal vessels systemically. They give short-term relief of congestion and are sometimes used to reduce ear pain and barotrauma during flying or diving, but they raise blood pressure and heart rate, keep patients awake, and have been linked to rare serious vascular brain events. They are not a long-term treatment.

## 1 CHAPTER ONE At a glance

The drugs on one page.

### 1.1 Summary

ORAL DECONGESTANTS

| | SUMMARY |
|---|---|
| **Agents** | Pseudoephedrine; phenylephrine (poorly absorbed, little better than placebo) |
| **How they work** | Sympathomimetic: release noradrenaline and stimulate alpha-adrenoceptors, constricting nasal venous sinusoids |
| **ENT uses** | Short-term congestion in colds and acute sinusitis; prevention of barotrauma during flights or dives in selected adults; Eustachian tube symptoms |
| **Adult dose** | Pseudoephedrine 60 mg every 4 to 6 hours (maximum 240 mg a day), short term; for flying, a dose about 30 to 60 minutes before descent |
| **Child dose** | Not recommended under 12 years in the UK |
| **Avoid** | Uncontrolled hypertension, severe kidney disease, ischaemic heart disease, within 2 weeks of an MAOI, phaeochromocytoma, hyperthyroidism, narrow-angle glaucoma |

# 2 CHAPTER TWO Using them in ENT

Indications.

## 2.1 Barotrauma and flying

Pseudoephedrine before descent reduces ear barotrauma in adults with a history of it, alongside autoinflation (Valsalva or a nasal balloon); it is not effective in children. For a diver, it can mask a problem that reappears at depth and is generally discouraged.

# 3 CHAPTER THREE Safety

What goes wrong.

## 3.1 Adverse effects

- Hypertension, tachycardia, palpitations, anxiety and **insomnia**; urinary retention.
- Rare **posterior reversible encephalopathy syndrome** and **reversible cerebral vasoconstriction syndrome** (sudden thunderclap headache): regulators have restricted their use.
- Misuse in illicit methamphetamine production explains sales controls.

## 3.2 Interactions

MAOIs and linezolid (hypertensive crisis), other sympathomimetics, beta blockers.

# 4 CHAPTER FOUR For the examination

What the examiner wants to hear.

## 4.1 Viva essentials

WHAT THE EXAMINER IS LOOKING FOR

- Cardiovascular contraindications.
- Their role in **barotrauma** prevention in adults.

## 4.2 Common pitfalls

COMMON PITFALLS

- Recommending pseudoephedrine to a hypertensive patient or a child.

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